A VISUAL GUIDE FOR NP STUDENTS

Topical corticosteroids, *decoded.*

One drug class. Seven potencies. Four textures. Here's how to think through matching the right steroid to the right skin — and the right amount.

1. **Potency**
   How powerful the anti-inflammatory effect is — matched to how severe and where the condition is.

2. **Vehicle**
   The cream, ointment, gel, or lotion base — matched to the skin's texture and location.

3. **Amount**
   How much to apply and for how long — enough to work, not enough to harm.

For the full drug and brand comparison list, use this alongside your required reading, the RxFiles *Topical Corticosteroids: Comparison Chart*.

DECISION ONE

The potency ladder

Topical steroids sit on a seven-step potency scale, usually grouped into four working bands. Climbing the ladder buys you speed and strength — and asks for more caution in return.

GROUP 1

Ultra-high potency

e.g. clobetasol propionate 0.05% · Max ≈ 50 g / week · course 2–4 weeks

Reserved for resistant, thickened, or severe disease — psoriasis, palms/soles, discoid lupus. Short, intermittent courses only; step down once controlled.

GROUPS 2–3

High potency

e.g. mometasone furoate ointment · Max ≈ 100 g / week

Moderate-to-severe eczema and psoriasis on the trunk and limbs. Still needs a defined course length, not open-ended use.

GROUPS 4–5

Mid potency

e.g. triamcinolone acetonide 0.1% · suitable for intermittent long-term use

A workhorse range for thicker-skinned areas — chronic hand eczema, stasis dermatitis, lichen sclerosus. Reasonable for on-and-off maintenance.

GROUPS 6–7

Low potency

e.g. hydrocortisone 1% · safest for sensitive sites

The default for face, eyelids, skin folds, genitals, infants, and older adults — and for maintenance once a flare is controlled.

DECISION TWO

Choosing the vehicle

Same drug, same potency, very different feel and behaviour depending on the base it's mixed into. Texture isn't cosmetic — it changes how the drug reaches the skin.



MOST OCCLUSIVE

Ointment

Greasy, seals in moisture. Best for dry, thick, or scaly skin — palms, soles, chronic plaques.

Avoid: skin folds & hairy areas — feels unpleasant, can macerate.

BALANCED

Cream

Cosmetically acceptable, less occlusive. Good all-rounder for wet or weepy lesions and skin folds.

Avoid: very dry, thickened skin — won't seal moisture in as well.

NON-GREASY

Gel

Dries quickly, leaves no residue. Useful on the scalp and other hairy areas.

Avoid: irritated or fissured skin — alcohol content can sting.

LEAST OCCLUSIVE

Lotion / foam

Spreads easily over large or hairy areas — scalp, groin, axilla — and acute weeping lesions.

Avoid: as a primary choice for thick, chronic plaques.

DECISION THREE

How much to apply

The standard unit for teaching dosing is the fingertip unit (FTU) — the ribbon of cream squeezed from the tip of an adult index finger to the first crease.

1 FTU ≈ 0.5 g

Covers roughly two adult palms of skin. Use it to teach patients a consistent, reproducible dose rather than a vague "thin layer."

BODY AREA	FTU / APPLICATION	GRAMS / APPLICATION	30-DAY SUPPLY, BID
One hand	1.0	0.5 g	30 g
One foot	2.0	1.0 g	60 g
Face & neck	2.5	1.25 g	75 g
One arm	3.0	1.5 g	90 g
Buttocks	4.0	2.0 g	120 g
One leg	6.0	3.0 g	180 g
Trunk, front or back	7.0	3.5 g	210 g
Entire body	40.0	20.0 g	1200 g

SCALING FOR CHILDREN

Adult FTU figures above need to be scaled down by age. Multiply the adult FTU by:

Infant (<1 yr)	× 0.2
Child	× 0.4
Adolescent	× 0.67

BEFORE YOU PRESCRIBE

Who needs extra caution

Infants & children

- More skin surface relative to body weight → absorb proportionally more drug
- Diapers and plastic pants trap heat and moisture, raising absorption up to 10×
- Default to low-potency agents whenever possible

Older adults

- Thinner, drier skin bruises and tears more easily
- Higher risk of atrophy, capillary fragility, and systemic absorption
- Low-potency ointments are often the safer starting point

Pregnancy & lactation

- Use the lowest effective potency for the shortest duration
- If a potent agent is truly needed, keep total course under ~300 g
- While breastfeeding, avoid high-potency product directly on the nipple

THE RULE THAT TIES IT TOGETHER

Use the least potent steroid that works, for the shortest time that works.

Potency, vehicle, and amount are all levers for the same goal: enough anti-inflammatory effect to control the skin condition, with the smallest possible dose of risk.

⚠ Don't stop high-potency steroids abruptly after prolonged use — taper gradually. Sudden discontinuation can trigger a withdrawal flare of redness and burning.

Built as a teaching aid for NP students. Always confirm current dosing, brand availability, and coverage against an up-to-date prescribing reference before patient use.

Companion resource to: L Regier, M Jin. *Topical Corticosteroids: Comparison Chart.* RxFiles Academic Detailing, University of Saskatchewan, 15th ed., April 2026. www.RxFiles.ca